

ENGLISH FOR WORK / SEPTEMBER 2026

Medical Assisting English



Learner workbook

Work with realistic cases, develop your own response, and use the answer section after you have tried the tasks.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For Medical assistants and outpatient clinic support staff.

Every patient, symptom report, date, measurement context, and clinic arrangement is fictional language practice. This is not clinical, diagnostic, triage, medication, specimen-collection, privacy-law, or treatment training. Duties depend on local rules, training, delegation, and supervision. Qualified clinicians assess urgency and make care decisions; use the workplace's actual emergency and escalation procedures, not these scripts, for real concerns.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

Open this course on English Ladder

Your course at a glance

Use the lesson numbers to match this document with the website and the other guides.

01 Clarifying the visit at check-in

Describe a visit mismatch without dismissing the patient's reason for attending.

02 Rooming and factual patient reports

Attribute symptom language accurately without diagnosing or minimizing.

03 Medication histories with clear sources

Identify conflicting medication information and request a clinician's review.

04 Laboratory paperwork and specimen queries

Distinguish collection, dispatch, and receipt in a laboratory enquiry.

05 Referrals and appointment dependencies

Explain completed referral steps and remaining scheduling questions.

06 Supporting questions during a clinical visit

Arrange a clinician response without giving instructions beyond your role.

07 Telephone concerns and clinical escalation

Decline an unsupported urgency judgment while arranging the correct response.

08 Records, corrections, and follow-up ownership

Repair a scheduling message while keeping result interpretation with the clinician.

Choose your pace

Quick practice: spend 15 minutes reading one conversation and rehearsing one scenario. Full lesson: allow 45-60 minutes for language work, three conversations, two speaking scenarios, and feedback.

Level support

B1 learners can use the frames and a partner. B2 learners can try a response before reading the model. C1 learners can add the harder follow-up and reduce preparation time. These are teaching suggestions, not certified CEFR level ratings.

After practicing: open the AI prompt workshop to choose a vocabulary, grammar, role-play, or new-dialogue activity.

LESSON 01 / READ AND NOTICE

Clarifying the visit at check-in

Describe a visit mismatch without dismissing the patient's reason for attending.

SITUATION

The schedule says annual review. Patient Alex expected a visit about a recent concern. The assistant cannot decide whether the appointment type is suitable. Nurse Dana can clarify the visit plan. Alex prefers Alex, while the verified record says Alexandra.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

chief concern - The main reason a patient gives for seeking care at the current visit.

intake form - A clinic document used to gather patient information before or during a visit.

patient identifier - A detail used with the clinic's verification process to distinguish one patient from another.

preferred name - The name a patient asks staff to use when addressing them.

Useful word combinations

confirm the visit purpose: check what the patient expects to discuss

verify patient identifiers: establish the correct record using the clinic's checks

clarify an intake response: ask what an unclear answer means

use the preferred name: address the patient as requested without confusing record identification

LESSON 01 / LANGUAGE AND PRACTICE

Acknowledge the expectation before checking the booking

3. Notice the language

A booking label and the patient's chief concern serve different purposes. Ask what the patient expected, preserve their wording, and leave decisions about the visit's clinical suitability to the team.

The schedule says ____; you expected ____.

How would you describe your main concern today?

I can address you as ____ while verifying the record under ____.

Compare this wording: You're booked for a review, so that's all we can discuss.

Clearer: The booking says review; I'll tell Dana about your additional concern so she can clarify the plan.

This checks the mismatch without making an unsupported scheduling or clinical decision.

4. Check the language

1. Patient: Alex is what I'd like you to call me.

Assistant: I'll use your ____, while checking the record separately.

- A. chief concern
- B. appointment type
- C. preferred name
- D. referral reason

2. Patient: The booking says annual review, but I expected to discuss a new concern. What happens now?

Assistant: I'll ask Dana ____ the visit plan before we confirm any change.

- A. to clarify
- B. clarified
- C. clarification
- D. clarifying

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 01 / 05 • CONVERSATIONS / 1 OF 3

Alex's visit expectation

Fictional check-in using the lesson case.

01 • Assistant: Alex, today's booking says annual review. What chief concern did you expect to discuss?

02 • Patient: Not exactly. I expected to discuss a recent concern, and I'm worried the booking is wrong.

03 • Assistant: I'll tell Dana about that difference so she can clarify the visit plan.

04 • Patient: Please don't assume I came for the wrong reason just because of the schedule label.

05 • Assistant: I won't. What would you like me to say is your main concern today?

06 • Patient: I'd like the clinician to hear about a change I noticed, rather than just review paperwork.

07 • Assistant: I'll preserve that request for Dana. I haven't confirmed whether the appointment type will change.

08 • Patient: Also, please call me Alex. Alexandra is on the record, but I don't use it socially.

09 • Assistant: I'll use Alex as your preferred name while keeping the verified record linked to Alexandra.

10 • Patient: Thank you. That sorts out the name question, and I'll discuss the visit plan with Dana.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 2 OF 3

The blank box

Fictional intake form has a blank contact field; the assistant checks its meaning privately.

- 01 • Assistant:** The emergency-contact field on your intake form is blank. Did you want help clarifying it?
- 02 • Patient:** I wasn't sure whether it asked for my number or someone else's emergency contact.
- 03 • Assistant:** It asks for an emergency contact. Your own telephone number is in the section above.
- 04 • Patient:** I understand now. I'd like to discuss whom to list before giving you a name.
- 05 • Assistant:** That's fine. I'll mark the field as not provided yet rather than assume nobody is available.
- 06 • Patient:** Does leaving it blank mean I can't have the conversation with the clinician today?
- 07 • Assistant:** I haven't been told that. I can ask reception about any remaining registration requirements.
- 08 • Patient:** Please ask them, but don't enter my sister automatically just because she brought me here.
- 09 • Assistant:** I won't infer your choice. The emergency-contact field remains open for you to clarify.
- 10 • Patient:** Good. I can decide whom to list after I understand what the clinic needs it for.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 3 OF 3

A quieter identity check

Fictional waiting room; patient requests privacy while identifiers are checked.

01 • Patient: Could we check my details somewhere quieter? I don't want everyone hearing my birth date.

02 • Assistant: Yes, the side desk is available. We'll continue your check-in there, away from the waiting chairs.

03 • Patient: Will you need the name from my record or the name I prefer people to use?

04 • Assistant: We need patient identifiers to verify the record, plus your preferred name for conversation.

05 • Patient: My record uses Samuel, but I prefer Sam. The surname is the same on both.

06 • Assistant: Thank you, Sam. We'll keep those purposes clear rather than create a second record for the name.

07 • Patient: I thought asking for a different name might mean starting all my registration again.

08 • Assistant: We can review your name preference within the existing verified record.

09 • Patient: That's helpful. I can answer the identity questions comfortably now that we're at this desk.

10 • Assistant: Let's continue here, and then I'll confirm your check-in status without announcing your private details outside.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Clarifying the visit at check-in

The schedule says annual review. Patient Alex expected a visit about a recent concern. The assistant cannot decide whether the appointment type is suitable. Nurse Dana can clarify the visit plan. Alex prefers Alex, while the verified record says Alexandra.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Alex: explain the different expectation and ask staff to use your preferred name.

Second round: Ask whether changing the display name also changes the verified identity record.

Scenario 2 | A form completed by a companion

Patient Noor's companion completed the intake form and selected routine follow-up. Noor says the main concern is different. No clinician has reviewed it. Nurse Dana is available to receive the discrepancy.

Role A: Assistant: ask Noor directly about the intended concern without criticizing the companion.

Role B: Noor: you want your own wording passed on and do not know the correct booking category.

Possible opening: My companion chose that box, but it isn't what I wanted to discuss.

Success checks

- Address Noor directly.
- Distinguish the form entry from the patient's current account.
- Give Dana the mismatch without deciding the clinical visit type.

Add a complication: The companion says the written answer should take priority over Noor's explanation.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 02 / READ AND NOTICE

Rooming and factual patient reports

Attribute symptom language accurately without diagnosing or minimizing.

SITUATION

During rooming, patient Pat reports tiredness since Monday. The assistant observed Pat answering questions while seated but has not assessed the cause or severity. Nurse Dana is receiving the report. No measurement values or clinical conclusions are supplied.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

rooming - Preparing a patient for the clinician's visit through the clinic's delegated intake activities.

vital signs - Basic physiological measurements, such as temperature and pulse, recorded for clinical review.

reported symptom - A bodily experience described by the patient, not necessarily observed by staff.

onset - The time or circumstances in which a reported symptom began.

Useful word combinations

record the patient's words: preserve the meaning and source of a symptom account

clarify symptom onset: ask when the patient says a change began

distinguish reported from observed: separate the patient's account from staff observations

relay a measurement query: pass uncertainty about a recorded measurement to the clinician

LESSON 02 / LANGUAGE AND PRACTICE

Separate a report from an interpretation

3. Notice the language

Use 'reports' for the patient's experience and 'I observed' for what you directly noticed. Neither phrase establishes a diagnosis. Time words such as 'since Monday' and 'on Monday' convey different durations.

The patient reports ___ since ___.

I observed ___ during ___.

The cause and ___ have not been established in this account.

Compare this wording: Pat is tired because of poor sleep.

Clearer: Pat reports tiredness since Monday; the cause has not been assessed in this exchange.

The new wording keeps the stated experience without adding a cause.

4. Check the language

1. Nurse: Did you see the tiredness begin on Monday?

Assistant: No. Pat ___ that it began then.

- A. reported
- B. diagnosed
- C. observed
- D. measured

2. Patient: It started Monday and is still happening.

Assistant: You have experienced it ___ Monday.

- A. since
- B. by
- C. on
- D. during

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 02 / 05 • CONVERSATIONS / 1 OF 3

What Pat reported

Fictional staff handoff using the lesson case, without clinical assessment.

01 • Assistant: Pat reports tiredness since Monday and wants to discuss that change with the clinician.

02 • Nurse: Which part did Pat tell you, and what did you observe yourself during rooming?

03 • Assistant: The reported symptom and onset came from Pat. I observed Pat answering questions while seated.

04 • Nurse: Have you established a cause, or is the reason for that change still unassessed?

05 • Assistant: I haven't established a cause or severity. I don't want my observation to imply either conclusion.

06 • Nurse: Did Pat describe it as constant, or have you only confirmed when it began?

07 • Assistant: Only the reported start on Monday. The pattern wasn't clarified in our conversation.

08 • Nurse: I'll take that distinction into the assessment. Please keep the missing detail explicit in your handoff.

09 • Assistant: Then the handoff is reported tiredness since Monday, answers while seated observed, pattern not clarified.

10 • Nurse: That's clear. I'll ask Pat about the unresolved details rather than infer them from the note.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 2 OF 3

Yesterday or last evening

Fictional patient clarifies the time meant by yesterday; no diagnostic conclusion.

01 • Assistant: You said the change began yesterday. Do you mean morning, afternoon, or evening?

02 • Patient: I meant after dinner last evening. I wasn't trying to say it lasted all day.

03 • Assistant: Thank you. I'll pass on after dinner yesterday evening rather than yesterday morning.

04 • Patient: I can't remember the exact hour. Is it a problem if I don't give a precise time?

05 • Assistant: You can say you don't know the exact time. I shouldn't supply one for you.

06 • Patient: Could you also tell the clinician it happened again this morning, not continuously overnight?

07 • Assistant: Yes, I'll preserve that distinction: last evening and again this morning, with the exact times uncertain.

08 • Patient: I don't know why it happened. Please don't write that I blamed the food.

09 • Assistant: I haven't added a cause. Your account describes timing, and the clinician can explore its meaning.

10 • Patient: That's what I wanted. I was using dinner to describe the time, not an explanation.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 3 OF 3

A measurement entry without its time

Fictional chart handoff; a vital-sign entry lacks a timestamp and its origin is unclear.

01 • Assistant: The vital signs entry lacks a timestamp. I can't tell whether it belongs to today's visit.

02 • Nurse: Did you take that measurement, or are you reviewing an entry already in the chart?

03 • Assistant: I'm reviewing an existing entry. I haven't recorded it as a measurement I obtained today.

04 • Nurse: Then please don't call it the current reading until we identify its source and time.

05 • Assistant: Could you review the entry while I explain the uncertainty to the rooming colleague?

06 • Nurse: Yes. I'll check the record history and determine what needs clarification for the visit.

07 • Assistant: The colleague asked whether the value means anything clinically. I haven't interpreted it.

08 • Nurse: Keep that question with the clinician. Our immediate documentation issue is when and where it originated.

09 • Assistant: I'll hand it over as an untimed existing entry, not a new observation from rooming.

10 • Nurse: That wording lets us resolve the source without turning an unknown timestamp into a clinical conclusion.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Rooming and factual patient reports

During rooming, patient Pat reports tiredness since Monday. The assistant observed Pat answering questions while seated but has not assessed the cause or severity. Nurse Dana is receiving the report. No measurement values or clinical conclusions are supplied.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Nurse Dana: ask which parts came from Pat and which were directly observed.

Second round: Ask whether answering questions proves the reported concern is minor.

Scenario 2 | The word dizzy needs the patient's explanation

A patient uses dizzy but then says they may mean tired. The assistant cannot interpret or assess the symptom. Nurse Dana is present and can take the concern now; no emergency symptoms are specified.

Role A: Assistant: ask the patient to describe the experience in their own words and pass it to Dana.

Role B: Patient: you are unsure which English word fits and want help communicating, not a diagnosis.

Possible opening: I said dizzy, but I'm not sure that is the right word in English.

Success checks

- Do not select a diagnosis or cause.
- Attribute the patient's wording and uncertainty.
- Connect the patient to Dana for assessment rather than deciding urgency from vocabulary.

Add a complication: A companion offers a cause the patient has not confirmed.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 03 / READ AND NOTICE

Medication histories with clear sources

Identify conflicting medication information and request a clinician's review.

SITUATION

Patient Ren says one medicine was stopped at another clinic. The local list still includes it. Ren does not have the other clinic's instruction. Nurse Dana can review the discrepancy with the clinician; the assistant cannot confirm discontinuation or change instructions.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

medication reconciliation - A clinician-led process of comparing medication information to identify and resolve differences.

medication list - A record of medicines reported or documented for a patient, with its source and review status.

nonprescription medicine - A medicine available without a prescription, also called an over-the-counter medicine.

allergy history - Recorded information about reported or established allergies, including details needing clinical review.

Useful word combinations

compare medication lists: identify differences between two stated sources

attribute a reported change: say who reported a medication change

flag an allergy-history discrepancy: draw attention to conflicting recorded allergy information

request clinician reconciliation: ask the responsible clinician to resolve medication-list differences

LESSON 03 / LANGUAGE AND PRACTICE

Ask for reconciliation, not a guessed correction

3. Notice the language

A patient report is important evidence to relay, not an instruction the assistant independently verifies. Keep source, date, and uncertainty visible. Do not infer a medicine from color or recommend changes.

The patient reports ____; the local list still shows ____.

We do not yet have ____ from ____.

Could you review this difference with ____?

Compare this wording: I'll remove it because you think the other clinic stopped it.

Clearer: I'll flag your report for the clinician; the current list still needs reconciliation.

This preserves the reported change without independently deciding the medication plan.

4. Check the language

1. Nurse: Has the other clinic confirmed the change?

Assistant: No. It is a ____ change from the patient.

A. prescribed

B. verified

C. reported

D. reconciled

2. Patient: I buy this medicine without a prescription. Should I mention it in the history?

Assistant: Yes, tell the clinician about the ____ medicine as well.

A. compounded

B. inpatient

C. nonprescription

D. discontinued

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 03 / 05 • CONVERSATIONS / 1 OF 3

A reported change from another clinic

Fictional medication-history exchange using the lesson case.

01 • Patient: The other clinic stopped one medicine, but it's still on the list you gave me.

02 • Assistant: I'll request medication reconciliation with the clinician. Do you have the other clinic's instruction?

03 • Patient: No, I remember the discussion, but I don't have a letter or updated list.

04 • Assistant: Then I'll attribute the change to your report and explain that confirmation is still missing.

05 • Patient: Can you simply take it off so we don't have to discuss it again?

06 • Assistant: The clinician needs to reconcile the difference. I can't settle the current medication plan myself.

07 • Patient: Please tell Dana I want the two lists compared, not another printed copy of this one.

08 • Assistant: I will. The request is to review the conflicting information and clarify the current list with you.

09 • Patient: Should I change what I take tonight while this is checked?

10 • Assistant: That's a medication question for the clinician. I'll include it explicitly in the handoff to Dana.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 2 OF 3

A supplement and an uncertain name

Fictional history conversation; the patient has an unlabeled memory, not a verified product name.

01 • Assistant: Are there other products you want the clinician to know about, including supplements or nonprescription medicines?

02 • Patient: I take a supplement, but I remember the package color better than the actual name.

03 • Assistant: I won't identify it from color. Can you provide the packaging name later?

04 • Patient: Yes, my partner can bring the package. I hadn't realized the name mattered to this conversation.

05 • Assistant: It helps the clinician review what you report. For now, I'll note that the name is unconfirmed.

06 • Patient: Please don't choose the product that sounds most similar from the list on your screen.

07 • Assistant: I won't. I'll say supplement reported, product name not yet provided, and packaging to follow.

08 • Patient: Could you ask the clinician whether they want to discuss it before I leave?

09 • Assistant: Yes, I'll include your question. I can't assess the supplement or recommend changes.

10 • Patient: I'll ask my partner to bring the packaging so the clinician has the actual name.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 3 OF 3

An allergy entry needs a source

Fictional chart query: blank allergy field and patient report require clinician review.

01 • Assistant: The allergy-history field is blank, but the patient reports a previous reaction they want to discuss.

02 • Nurse: A blank field doesn't establish no allergies. What did the patient actually tell you?

03 • Assistant: They remember a reaction but are unsure of the medicine name and the date.

04 • Nurse: Please preserve that uncertainty and bring the question to me for clinical review.

05 • Assistant: Should I call it a confirmed allergy before you've spoken with the patient?

06 • Nurse: No. Attribute it to the patient, and don't turn the blank field into a negative finding either.

07 • Assistant: I'll say reaction reported, medicine and date uncertain, allergy history awaiting clinician clarification.

08 • Nurse: That's specific. Have you offered any explanation of what the reaction means or how to manage it?

09 • Assistant: No. I told the patient you would discuss it with them, without interpreting the experience.

10 • Nurse: Thank you. I'll take the history question now and address the unresolved details directly.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Medication histories with clear sources

Patient Ren says one medicine was stopped at another clinic. The local list still includes it. Ren does not have the other clinic's instruction. Nurse Dana can review the discrepancy with the clinician; the assistant cannot confirm discontinuation or change instructions.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Ren: explain that another clinic discussed a change, but you do not have its instructions.

Second round: Ask the assistant whether to take the medicine tonight; redirect the question to the clinician.

Scenario 2 | Two differently dated lists

A patient brings a list dated June and a phone photograph dated August. One item differs. Neither has been reconciled with today's chart. Nurse Dana can review both with the clinician.

Role A: Assistant: identify the sources and date difference.

Role B: Patient: ask which version is correct; you do not know why they differ.

Possible opening: The photograph is newer, so can you assume everything on it is current?

Success checks

- Name the two dated sources.
- Do not equate newer with clinically reconciled.
- Ask the clinician to review the differences without changing instructions.

Add a complication: The patient asks the assistant to delete every item missing from the photograph.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 04 / READ AND NOTICE

Laboratory paperwork and specimen queries

Distinguish collection, dispatch, and receipt in a laboratory enquiry.

SITUATION

A requisition gives 09:20 as collection time; the courier sheet says 10:05 arrival. A colleague proposes replacing 09:20 with 10:05. Both times describe different events. No identity discrepancy or test result is stated.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

specimen - A sample of material from a patient submitted for examination or testing.

requisition - A document or electronic request specifying the laboratory testing requested by a clinician.

collection time - The recorded time when a specimen was obtained, distinct from receipt at the laboratory.

specimen identifier - A label or code linking a specimen to the appropriate patient and order.

Useful word combinations

match the requisition: compare the requested test details with the accompanying record

clarify the collection time: obtain the time of sampling rather than delivery

flag a label mismatch: report disagreement between identifiers for review

confirm laboratory receipt: establish that the receiving laboratory has logged the specimen

LESSON 04 / LANGUAGE AND PRACTICE

Clarify which event a timestamp describes

3. Notice the language

An accurate time paired with the wrong event is misleading. Ask who recorded the time and what happened then. This lesson does not teach specimen collection, labeling, handling, acceptance, or testing procedures.

Does ___ refer to collection or receipt?

The requisition records ___; the courier sheet records ___.

Which source confirms ___?

Compare this wording: The sample time is ten oh five.

Clearer: Ten oh five is courier arrival; the recorded collection time is nine twenty.

The revision attaches each time to the correct event.

4. Check the language

1. Colleague: Which time tells us when the sample was obtained, not when it arrived?

Assistant: The ___ time.

A. reporting

B. receipt

C. dispatch

D. collection

2. Laboratory: We logged arrival, but suitability review is incomplete.

Assistant: Receipt is confirmed; acceptance is still ____.

A. pending

B. completed

C. withdrawn

D. duplicated

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 04 / 05 • CONVERSATIONS / 1 OF 3

Two times, two events

Fictional paperwork discussion using the lesson case.

01 • Assistant: You suggested changing the collection time to ten oh five. What does that later time describe?

02 • Colleague: It's the courier arrival time. I thought the most recent timestamp should replace the older one.

03 • Assistant: The requisition records collection at nine twenty. Arrival and collection are different events, not competing versions.

04 • Colleague: So the difference doesn't automatically mean someone entered a wrong time on the requisition?

05 • Assistant: Correct. The records can both be accurate because they describe different points in the specimen journey.

06 • Colleague: I'll keep each time with its label. What if someone questions the collection entry itself?

07 • Assistant: Ask the recording team about that entry specifically, without substituting the arrival time.

08 • Colleague: Does the arrival record tell us that the specimen has been accepted for testing?

09 • Assistant: No. Receipt and acceptance are different statuses; we'd need the laboratory's confirmation of acceptance.

10 • Colleague: I'll report collection at nine twenty and arrival at ten oh five, with acceptance unstated.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 2 OF 3

A request number read back

Fictional clinic-laboratory call; two similar request references need clarification.

01 • Assistant: I'm checking receipt for request L36, but the message I received refers to L63.

02 • Laboratory clerk: Please repeat the reference from your requisition slowly so I can check the correct entry.

03 • Assistant: The requisition says L36. The reply says L63, with those two digits reversed.

04 • Laboratory clerk: Thank you. I can see L36 now; its receipt is recorded, but no result is listed.

05 • Assistant: Then your confirmation concerns L36's receipt, not L63 or a completed test result.

06 • Laboratory clerk: Exactly. I'll correct the reference in my reply so the two enquiries aren't joined together.

07 • Assistant: Could you also identify receipt explicitly? Our clinician asked whether the laboratory had the specimen.

08 • Laboratory clerk: Yes. The reply will say receipt confirmed for L36, with result status not available here.

09 • Assistant: That's what we needed. I won't call the specimen lost based on that reference mismatch.

10 • Laboratory clerk: Thank you for checking. The corrected reference should let your team attach the reply to the right enquiry.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 3 OF 3

The label question before a conclusion

Fictional specimen paperwork concern: identifiers differ; the laboratory lead must review.

01 • Assistant: The requisition and specimen identifier don't agree. I'm reporting that mismatch before describing test status.

02 • Laboratory lead: Which sources differ, and has anyone confirmed that the sample belongs to the requisition you have?

03 • Assistant: No match has been confirmed. I'm reporting the difference, not deciding which source is correct.

04 • Laboratory lead: Thank you. I'll handle the review with the responsible team; don't resolve identity from memory.

05 • Assistant: The clinician is asking when the result will appear. What can I say about that?

06 • Laboratory lead: Say an identifier discrepancy is under review and no completion time has been established.

07 • Assistant: Has rejection been confirmed, or is that decision still pending?

08 • Laboratory lead: No acceptance or rejection decision has been communicated. Keep those statuses distinct from the review.

09 • Assistant: I'll give that update and identify you as the person reviewing the mismatch with the team.

10 • Laboratory lead: Please do. I'll return the verified status through the clinic's laboratory communication route when available.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Laboratory paperwork and specimen queries

A requisition gives 09:20 as collection time; the courier sheet says 10:05 arrival. A colleague proposes replacing 09:20 with 10:05. Both times describe different events. No identity discrepancy or test result is stated.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Colleague: you think the later courier time should replace the earlier requisition time.

Second round: Ask whether laboratory receipt guarantees that the specimen has been accepted for testing.

Scenario 2 | Preparation wording needs clarification

A patient has an appointment for a clinician-ordered test but received two different preparation messages. Their exact medical details are not supplied. Nurse Dana can compare the messages and obtain clinical clarification.

Role A: Assistant: identify that there are two messages without choosing instructions.

Role B: Patient: ask which message to follow and whether the appointment is cancelled.

Possible opening: One message says something different from the other. Which preparation instruction is right?

Success checks

- Request the actual messages for clinician review.
- Do not invent preparation directions.
- Keep clarification distinct from cancellation or confirmed readiness.

Add a complication: The patient asks you to use the newer message automatically.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 05 / READ AND NOTICE

Referrals and appointment dependencies

Explain completed referral steps and remaining scheduling questions.

SITUATION

Referral F8 was sent Monday and received Tuesday. The specialist office says it is under review, with no appointment booked. Coordinator Lia has agreed to check the status Friday. The patient believes receipt means a Friday appointment.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

referral - A clinician's request for another service or professional to assess or manage a patient's care.

specialist - A clinician whose practice focuses on a particular field of healthcare.

appointment confirmation - A notice establishing a booked visit's agreed date, time, and location.

referral status - The recorded stage of a referral, such as sent, received, under review, or scheduled.

Useful word combinations

track a referral: check its progress through the receiving service

confirm receipt with the specialist office: establish that the request arrived

distinguish review from booking: avoid treating request assessment as an appointment

verify the clinic location: check which site is named in the appointment confirmation

LESSON 05 / LANGUAGE AND PRACTICE

Separate referral progress from a booked visit

3. Notice the language

Sent, received, reviewed, and scheduled name distinct stages. Give the known stage and next contact without predicting the specialist's decision or timing.

The referral was ___ on ___; it is now ___.

___ is our status-check date, not ___.

We still need confirmation of ___ before ___.

Compare this wording: The referral arrived, so you're booked for Friday.

Clearer: The referral is under review. Friday is Lia's next status check; no visit is booked.

A follow-up date should not be mistaken for an appointment.

4. Check the language

1. Patient: Is Friday my specialist visit?

Assistant: No. Friday is the date Lia will ___ the referral status.

- A. attend
- B. check
- C. complete
- D. cancel

2. Specialist office: We received it, but no visit is booked.

Assistant: I'll report received, not ____.

- A. submitted
- B. scheduled
- C. requested
- D. dispatched

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 05 / 05 • CONVERSATIONS / 1 OF 3

Friday is a check, not a visit

Fictional referral update using the lesson case.

01 • Patient: I took Friday off for the specialist. What time should I arrive there?

02 • Assistant: Friday is Lia's status check, not a booked visit. I'm sorry that distinction wasn't clear.

03 • Patient: But the specialist office received the referral Tuesday. Doesn't that mean they accepted an appointment?

04 • Assistant: Receipt means it arrived. They say it's under review, with no appointment booked.

05 • Patient: Was sending it Monday a separate step, or did Tuesday replace that date?

06 • Assistant: Both are relevant: sent Monday, received Tuesday. Neither date is a visit confirmation.

07 • Patient: Please tell Lia I've arranged leave and need the actual date before making travel plans.

08 • Assistant: I'll pass that on without promising the review will finish before Friday.

09 • Patient: Should I call the specialist to ask which site I should attend anyway?

10 • Assistant: We don't have a booked site yet. The appointment confirmation will need both location and time.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 2 OF 3

The same clinic name at two sites

Fictional confirmation lists East Clinic; patient assumes the West site.

01 • Assistant: Your confirmation says East Clinic on Thursday at ten. Which site were you planning to visit?

02 • Patient: West Clinic, because that's where I went last year for another appointment.

03 • Assistant: This confirmation names East. Last year's location doesn't establish the site for Thursday.

04 • Patient: Could you read the street name from the confirmation so I can check transport?

05 • Assistant: It says Lake Street. I'll repeat the full location from your current confirmation privately.

06 • Patient: Thank you. I thought East and West were departments in the same building.

07 • Assistant: They are listed as different sites here. I won't describe them as one building.

08 • Patient: Does checking the address change the time or create a second appointment for me?

09 • Assistant: No, we are clarifying the existing Thursday booking, not changing it or creating another.

10 • Patient: Then I'll plan for East Clinic on Lake Street at ten, as this confirmation says.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 3 OF 3

Receipt not found at the receiving office

Fictional referral has a sent record but no acknowledgment.

01 • Assistant: Our record says referral H4 was sent yesterday, but the specialist office can't find it.

02 • Referral coordinator: Do we have an acknowledgment, or only the record of our outgoing submission?

03 • Assistant: Only the outgoing record. I haven't described receipt as confirmed to the patient.

04 • Referral coordinator: I'll check the destination details and contact the receiving office about that reference.

05 • Assistant: Could you tell me what wording to use while your delivery enquiry remains open?

06 • Referral coordinator: Say we have a sent record but haven't confirmed receipt. Avoid calling the referral reviewed.

07 • Assistant: The patient asks whether a new referral is required. No clinician has decided that.

08 • Referral coordinator: Keep that decision separate. We're first checking where the existing request went.

09 • Assistant: I'll explain the delivery enquiry and say you are coordinating it with the receiving office.

10 • Referral coordinator: Please do. I'll report what they confirm rather than simply resend and assume the problem is solved.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Referrals and appointment dependencies

Referral F8 was sent Monday and received Tuesday. The specialist office says it is under review, with no appointment booked. Coordinator Lia has agreed to check the status Friday. The patient believes receipt means a Friday appointment.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Patient: you arranged leave Friday because you confused Lia's check with a specialist visit.

Second round: Ask whether under review means the specialist has accepted the referral.

Scenario 2 | A cancellation list is not a booking

A specialist office offers a cancellation list while the patient's existing appointment remains next month. No earlier slot is available. Lia can explain the administrative distinction.

Role A: Assistant: explain the existing booking and possible earlier opportunity.

Role B: Patient: you think joining the list cancels next month's appointment.

Possible opening: If I join that list, do I lose the appointment I already have?

Success checks

- Preserve the existing appointment in the explanation.
- Describe an earlier slot as unconfirmed.
- Avoid predicting priority or promising a clinical scheduling decision.

Add a complication: The patient asks for a guaranteed earlier slot before changing work plans.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 06 / READ AND NOTICE

Supporting questions during a clinical visit

Arrange a clinician response without giving instructions beyond your role.

SITUATION

Patient Jules does not understand a phrase in the after-visit summary and has not left. Dr Chen is available. The assistant cannot reinterpret the instruction. Jules wants an explanation before leaving, away from reception.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

chaperone - A person present during an examination according to the patient's needs and the clinic's arrangements.

consent discussion - A clinician-led exchange about a proposed examination or treatment and the patient's decision.

after-visit summary - A document summarizing information and instructions from a clinical encounter.

teach-back - An invitation for a patient to explain their understanding so the clinician can check the explanation.

Useful word combinations

invite a question: create space for a patient to raise an uncertainty

request a pause: communicate that the patient wants a discussion or activity stopped temporarily

clarify the after-visit summary: ask the clinician to explain unclear visit documentation

check understanding through teach-back: invite a restatement without testing or blaming the patient

LESSON 06 / LANGUAGE AND PRACTICE

Preserve the question and the patient's preference

3. Notice the language

Identify the unclear phrase and requested conversation setting. A clarification question is not refusal of all care. Teach-back checks the explanation, not the patient's intelligence.

The patient wants ___ clarified before ___.

I haven't changed or interpreted ___.

Could you return to ___ and check ___?

Compare this wording: It just means the usual thing; you can go now.

Clearer: That phrase is unclear to you. I'll ask Dr Chen to explain it here before you leave.

The question gets a specific recipient and location.

4. Check the language

1. Patient: I nodded, but I still have a question about this phrase.

Assistant: I'll ask Dr Chen to ___ it with you.

A. abbreviate

B. file

C. withdraw

D. clarify

2. Clinician: I want to check whether my explanation was clear.

Assistant: Inviting the patient to explain it in their own words is ____.

A. a referral

B. discharge approval

C. a diagnosis

D. teach-back

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 06 / 05 • CONVERSATIONS / 1 OF 3

An unclear phrase before leaving

Fictional visit-room exchange using the lesson case.

01 • Patient: I don't understand this phrase in the after-visit summary, though I nodded when the clinician explained.

02 • Assistant: Thank you for telling me. Which phrase would you like Dr Chen to discuss again?

03 • Patient: This one here. I don't want you to guess; I need to know what was intended.

04 • Assistant: I'll ask Dr Chen to return. I haven't interpreted or changed the instruction.

05 • Patient: Could we talk here rather than at reception? There are people I know outside.

06 • Assistant: Yes, I'll include that preference. Dr Chen is available, and I'll ask for the conversation here.

07 • Patient: Does asking again mean you will record that I refused the whole plan?

08 • Assistant: A clarification question isn't the same as refusing everything. I'll describe the question you actually raised.

09 • Patient: Please say I want an explanation before leaving, not another copy of the same sheet.

10 • Assistant: That's the handoff I'll give: this phrase needs explanation here, before you leave the clinic.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 2 OF 3

A pause is a request, not a motive

Fictional patient pauses an examination discussion; no procedure is described.

01 • Patient: Could we pause the discussion? I'm trying to understand what the clinician just said.

02 • Assistant: I'll let the clinician know you want a pause and another explanation.

03 • Patient: Please don't say I'm refusing everything. I haven't made that decision yet.

04 • Assistant: I won't infer a decision. You are asking for time and clarification at this point.

05 • Patient: My companion answers quickly, but I need the clinician to ask me directly.

06 • Assistant: I'll communicate that too. Would you like me to use those exact words?

07 • Patient: Yes: please ask me directly, and give me time before moving on.

08 • Assistant: That's clear. I can relay that request without deciding what you will agree to.

09 • Patient: Thank you. I was worried asking for a pause would sound rude in English.

10 • Assistant: It is a clear request. I'll bring the clinician back into the conversation with you now.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 3 OF 3

A chaperone request with a question

Fictional chaperone enquiry; arrangements and role need explanation.

01 • Patient: I saw the word chaperone on the form. Is that the same as my companion?

02 • Assistant: Not necessarily. The clinic can explain the chaperone's role and the arrangements for your visit.

03 • Patient: Could someone explain who that person would be and whether I can ask questions?

04 • Assistant: I'll ask the clinician to discuss that with you before you decide about the examination.

05 • Patient: I don't want my question recorded as agreement just because I asked what the word means.

06 • Assistant: Of course. Asking for an explanation doesn't by itself establish agreement to an examination.

07 • Patient: Please tell the clinician I'm comfortable discussing the options, but haven't decided yet.

08 • Assistant: I'll preserve that distinction. I haven't marked your question as a completed consent discussion.

09 • Patient: Will the clinician also explain what my companion can do during the conversation?

10 • Assistant: Yes, I'll include that as a separate question about your companion's role and your preferences.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Supporting questions during a clinical visit

Patient Jules does not understand a phrase in the after-visit summary and has not left. Dr Chen is available. The assistant cannot reinterpret the instruction. Jules wants an explanation before leaving, away from reception.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Jules: point out an unclear phrase without inventing its clinical meaning.

Second round: Say you nodded to be polite but still do not understand.

Scenario 2 | A summary from an earlier visit

A patient opens a portal document dated last month. Today's summary has not appeared. Dr Chen can clarify today's information before the patient leaves.

Role A: Assistant: identify the date mismatch and request clarification.

Role B: Patient: you want to avoid relying on the wrong document.

Possible opening: Why does this document show last month's date?

Success checks

- Name the date mismatch.
- Do not adapt clinical instructions.
- Ask Dr Chen to clarify today's summary and guidance.

Add a complication: The patient asks you to adapt the older instructions to today's visit.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 07 / READ AND NOTICE

Telephone concerns and clinical escalation

Decline an unsupported urgency judgment while arranging the correct response.

SITUATION

A caller asks the assistant to decide whether a newly reported concern can wait until next week's routine visit. No clinical details or urgency assessment are supplied. Nurse Dana is available now through the clinic's clinical-call route. The assistant cannot triage.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

triage - Clinical assessment used to determine the urgency and appropriate response to a healthcare concern.

escalation - Bringing a concern to a person or pathway with the required responsibility to respond.

callback number - The verified telephone number to use when returning a particular call.

warm transfer - A telephone handoff in which the sender introduces the caller's issue to the receiving person.

Useful word combinations

route a clinical concern: connect the caller with the appropriate clinical response

verify the callback number: check the number for this particular return contact

make a warm transfer: introduce the concern before handing over the call

state the caller's exact request: preserve what the person wants assessed or explained

LESSON 07 / LANGUAGE AND PRACTICE

Do not let a booking answer a clinical question

3. Notice the language

A routine appointment label does not assess a new concern. Use actual local emergency and escalation procedures for real calls. This exercise supplies no symptom criteria and teaches no triage decisions.

I can't determine ___ from ___.

The question for the nurse is ___.

I'll connect you with ___ and identify ___.

Compare this wording: You're booked next week, so it can wait.

Clearer: The booking doesn't assess this new concern. I'll connect you with Dana for clinical review.

A calendar entry cannot establish clinical urgency.

4. Check the language

1. Caller: Does having a routine appointment mean this new concern is routine?

Assistant: No. The new concern needs clinical ____.

A. billing

B. confirmation of attendance

C. registration

D. triage

2. Nurse: Will you introduce the caller's question before connecting us?

Assistant: Yes, I'll make a ____.

A. reminder notice

B. missed call

C. booking amendment

D. warm transfer

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 07 / 05 • CONVERSATIONS / 1 OF 3

A booking cannot assess the concern

Fictional telephone exchange using the lesson case.

01 • Caller: I have a routine appointment next week. Can this new concern wait until then?

02 • Assistant: I can't determine that from the booking. Dana can assess your concern through our clinical-call route now.

03 • Caller: Could you just say yes or no? I don't want to explain everything twice.

04 • Assistant: I can introduce the question to Dana, but I can't perform clinical triage myself.

05 • Caller: Please don't describe me as asking to change an appointment. I'm asking about the concern itself.

06 • Assistant: I'll make that distinction. You want clinical advice about timing, not just another calendar slot.

07 • Caller: Is Dana actually available, or are you placing me in an ordinary booking queue?

08 • Assistant: Dana is available now for this handoff. I'll introduce the question before connecting you.

09 • Caller: All right. Tell her I need to know how to respond to the new concern.

10 • Assistant: I will. I haven't characterized it as routine, and Dana will take the clinical conversation.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 2 OF 3

The number for this call

Fictional administrative callback: patient will be away from their usual telephone; no symptoms.

01 • Assistant: Reception will return your scheduling call this afternoon. Is the usual number suitable today?

02 • Patient: Not today. I'll be at my sister's home and want the call on my mobile.

03 • Assistant: Let's verify the callback number privately. Should this change apply only to today's call?

04 • Patient: Yes, only today. Please don't replace my permanent home number because of one afternoon.

05 • Assistant: I'll distinguish today's callback number from a permanent profile change. Which ending should we verify?

06 • Patient: The mobile ends in sixty-eight. Please compare the full number with me here privately.

07 • Assistant: We have checked it. Reception's task is to discuss scheduling, not give clinical advice.

08 • Patient: That's correct. I'm asking whether a later arrival time is available for the same visit.

09 • Assistant: I'll give reception that request and today's verified callback number, with availability still unconfirmed.

10 • Patient: Thank you. That avoids a call to an empty house and keeps my usual contact details unchanged.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 3 OF 3

A nurse accepts the live call

Fictional internal telephone introduction; no symptom assessment or care instruction.

01 • Assistant: I have a caller asking whether a newly reported concern can wait for next week's appointment.

02 • Nurse: I'll take that clinical conversation. Have you offered reassurance or assigned it a routine priority?

03 • Assistant: No. I said the appointment label doesn't assess the concern, and I haven't interpreted it.

04 • Nurse: Good. Please introduce me while the caller is connected so they know I've accepted the handoff.

05 • Assistant: Could I summarize the question during a warm transfer, before you ask further details?

06 • Nurse: Yes. Summarize the request, but I'll need to hear the clinical information directly.

07 • Assistant: They want advice on timing for a new concern, not simply a different appointment slot.

08 • Nurse: That's the distinction I need. I am accepting the call now, not merely a message for later.

09 • Assistant: I'll connect the caller and say you're taking the clinical discussion from this point.

10 • Nurse: Thank you. Scheduling remains separate until the clinical response establishes what arrangements are needed.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Telephone concerns and clinical escalation

A caller asks the assistant to decide whether a newly reported concern can wait until next week's routine visit. No clinical details or urgency assessment are supplied. Nurse Dana is available now through the clinic's clinical-call route. The assistant cannot triage.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Caller: ask whether a new concern can wait; no symptom details are supplied in this exercise.

Second round: Ask for a yes-or-no answer because you do not want to repeat the question.

Scenario 2 | A disconnected handoff

A scheduling caller is disconnected before a warm transfer reaches reception. Reception has not accepted the call. The assistant has the caller's verified number and can return the call now. No clinical concern is involved.

Role A: Assistant: clarify the failed handoff and re-establish contact.

Role B: Reception colleague: you did not receive the caller and can take the call after reconnection.

Possible opening: The transfer disconnected before you answered; can you take the scheduling call once I reconnect?

Success checks

- Separate a transfer attempt from accepted contact.
- Use the verified callback number without inventing an outcome.
- Confirm who is reconnecting and who will receive the call.

Add a complication: The task display says transferred, and a colleague assumes the conversation occurred.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 08 / READ AND NOTICE

Records, corrections, and follow-up ownership

Repair a scheduling message while keeping result interpretation with the clinician.

SITUATION

The assistant's message incorrectly says a results discussion is booked Tuesday. The schedule confirms Thursday at 14:00 with Dr Chen. The patient rearranged work. The assistant can correct the message and confirm the booking; no results are discussed in this exercise.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

electronic health record - The clinic's digital record of patient information, encounters, and related care documentation.

amendment - A traceable correction or addition to an existing record made through the required process.

release of information - The provision of patient records or details to a recipient after applicable permission and identity checks.

follow-up task - An assigned action still needed after an encounter, message, or review.

Useful word combinations

request a record amendment: raise a specific correction for review

confirm the release recipient: establish who is to receive the requested information

trace a contact attempt: identify whether and how staff tried to reach a patient

accept follow-up ownership: explicitly take responsibility for the remaining action

LESSON 08 / LANGUAGE AND PRACTICE

Correct the information and acknowledge its impact

3. Notice the language

State what was wrong and what the verified record says. An amendment should remain traceable. Updating a record, reaching the patient, and finishing a follow-up task are different actions.

My earlier message incorrectly said ____.

The confirmed information is ____.

The correction is recorded; ____ still needs to happen.

Compare this wording: Sorry for the confusion; check the portal.

Clearer: I gave the wrong day. Your confirmed discussion is Thursday at two with Dr Chen.

A specific correction is more useful than a vague apology.

4. Check the language

1. Patient: Which day is correct?

Assistant: My message was wrong. The ____ appointment is Thursday at two.

- A. cancelled
- B. provisional
- C. requested
- D. confirmed

2. Colleague: The note is corrected, but the patient has not been reached.

Assistant: The contact task is still ____.

- A. outstanding
- B. completed
- C. duplicated
- D. withdrawn

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 08 / 05 • CONVERSATIONS / 1 OF 3

Tuesday was the incorrect message

Fictional correction call using the lesson case.

01 • Assistant: My message gave Tuesday incorrectly. Your electronic health record confirms Thursday at two with Dr Chen.

02 • Patient: I changed my work shift because of Tuesday. Why should I trust this new date?

03 • Assistant: I've checked the appointment schedule directly. Thursday at two is the confirmed entry.

04 • Patient: Please correct the earlier message so reception doesn't repeat Tuesday if I call again.

05 • Assistant: I will, using the record's correction process. I understand the wrong date disrupted your work plans.

06 • Patient: Does the change mean the results are less important than you first thought?

07 • Assistant: This is a correction to my scheduling message, not a clinical judgment about the results.

08 • Patient: Then I need the clinician to explain them during the Thursday discussion, not over this call.

09 • Assistant: That's right. I can confirm the booking, but I won't interpret the results.

10 • Patient: Please send the corrected appointment details through my usual contact route so I can rearrange work accurately.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 2 OF 3

A record request with an unclear recipient

Fictional records enquiry; patient asks for a summary to be sent but the destination is unspecified.

01 • Patient: Could you send my visit summary to the other office? They said they need it.

02 • Assistant: Which office do you mean? We need the intended recipient and the required release checks.

03 • Patient: The specialist office, not my employer. I should have been more specific about that.

04 • Assistant: I'll pass the release of information request to our records team, identifying the specialist office.

05 • Patient: Does asking you now mean the summary has already been sent to someone?

06 • Assistant: No. This is a request awaiting the records team's checks, not a completed release.

07 • Patient: Could they confirm the destination with me before describing the task as finished?

08 • Assistant: I'll include that request. We haven't established the full recipient details in this conversation.

09 • Patient: Good. I don't want the phrase other office copied into a message as if it identified anyone.

10 • Assistant: I'll describe the specialist office as intended, with the exact recipient still to be confirmed.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 3 OF 3

A reached patient or an unanswered call

Fictional follow-up record says contacted, but only an unanswered attempt occurred.

01 • Assistant: The follow-up entry says contacted, but I only made an unanswered call this morning.

02 • Team lead: Then contacted is too definite. Did you leave a message or speak with anyone?

03 • Assistant: No one answered, and I didn't leave a message. The conversation has not occurred.

04 • Team lead: Please make a traceable amendment and keep the follow-up task open until contact occurs.

05 • Assistant: I'll describe an unanswered attempt at ten, rather than a completed discussion with the patient.

06 • Team lead: Who owns the next contact attempt? A corrected note alone doesn't settle that responsibility.

07 • Assistant: I can own it this afternoon, using the verified contact preference already in the profile.

08 • Team lead: That is clear. Don't imply the patient declined to speak just because the call wasn't answered.

09 • Assistant: I won't assign a reason. I'll record the attempt and the actual next-contact outcome.

10 • Team lead: Please let me know if contact remains unsuccessful so we can review the outstanding task.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Records, corrections, and follow-up ownership

The assistant's message incorrectly says a results discussion is booked Tuesday. The schedule confirms Thursday at 14:00 with Dr Chen. The patient rearranged work. The assistant can correct the message and confirm the booking; no results are discussed in this exercise.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Patient: you rearranged work based on Tuesday and want the verified details.

Second round: Ask whether the wrong date says anything about the clinical importance of the results.

Scenario 2 | A copied appointment time

An old message says 11:00. Today's confirmed booking is 11:30 at the same clinic. A colleague copied the old message into a reminder but has not sent it. You can prevent the inaccurate reminder now.

Role A: Assistant: identify the outdated source and request a corrected reminder.

Role B: Colleague: ask whether the clinic location also changed; it did not.

Possible opening: The unsent reminder still says eleven, but today's booking confirms eleven thirty.

Success checks

- Correct only the verified time difference.
- Keep the unchanged location explicit.
- Confirm the corrected reminder before describing it as sent.

Add a complication: The colleague assumes every detail from the old message must be wrong.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

Answer workshop

Try the tasks first. The model is one acceptable response to scenario 1, not a script to memorize. Assess your spoken version using the four criteria at the end.

01 | Clarifying the visit at check-in

Editing example: The booking says review; I'll tell Dana about your additional concern so she can clarify the plan.

This checks the mismatch without making an unsupported scheduling or clinical decision.

Check 1: C - preferred name

A: Chief concern is the reason for seeking care.

B: Appointment type describes the booked visit.

C: Correct. Preferred name is the requested form of address.

D: Referral reason concerns why another service was requested.

Check 2: A - to clarify

A: Correct. Ask Dana to clarify uses the request pattern ask someone to do something.

B: Clarified is a past form; this request needs to plus the base verb.

C: Clarification is a noun, but the gap needs a verb describing Dana's requested action.

D: Clarifying is an -ing form; ask someone requires to plus a verb here.

Model response: Alex, the schedule describes an annual review, but you expected to discuss a recent concern. I will tell Dana about that difference so she can clarify the visit plan with you. I can use Alex when addressing you while keeping the verified record linked to Alexandra. I haven't confirmed a change to the appointment type.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

02 | Rooming and factual patient reports

Editing example: Pat reports tiredness since Monday; the cause has not been assessed in this exchange.

The new wording keeps the stated experience without adding a cause.

Check 1: A - reported

A: Correct. Reported attributes the onset to Pat's account.

B: Diagnosed means identified a condition, which Pat did not do here.

C: Observed would attribute a direct observation rather than a symptom report.

D: Measured requires an actual measurement, not this account.

Check 2: A - since

A: Correct. Since links a starting point to continuing experience.

B: By gives a deadline or latest point, not this duration.

C: On identifies a day without expressing continuation to now.

D: During refers to a period rather than this starting point.

Model response: Pat reports feeling tired since Monday. During our conversation, Pat answered questions while seated. That observation does not explain the tiredness or establish its severity. Dana, I am passing on the reported onset and what I directly observed; Pat wants to discuss the change with the clinician. I have not offered an explanation for it.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

03 | Medication histories with clear sources

Editing example: I'll flag your report for the clinician; the current list still needs reconciliation.

This preserves the reported change without independently deciding the medication plan.

Check 1: C - reported

A: Prescribed implies a current authorized instruction.

B: Verified falsely says confirmation occurred.

C: Correct. Reported identifies the source without verification.

D: Reconciled would imply that differences were resolved.

Check 2: C - nonprescription

A: Compounded describes preparation, not whether a prescription is needed.

B: Inpatient describes a care setting, not purchase without a prescription.

C: Correct. Nonprescription names the category described.

D: Discontinued means stopped, which is not stated.

Model response: Dana, could you review this medication-list discrepancy with the clinician? Ren reports that another clinic stopped one medicine, but our list still includes it and we do not have that instruction. Please clarify the current list with Ren. I have recorded the report as unverified and have not advised starting, stopping, or changing anything.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

04 | Laboratory paperwork and specimen queries

Editing example: Ten oh five is courier arrival; the recorded collection time is nine twenty.

The revision attaches each time to the correct event.

Check 1: D - collection

A: Reporting concerns communication of test information.

B: Receipt concerns arrival at the receiving location.

C: Dispatch concerns sending the specimen onward.

D: Correct. Collection is when the sample was obtained.

Check 2: A - pending

A: Correct. Pending marks the unfinished review.

B: Completed contradicts the stated incomplete review.

C: Withdrawn would mean removed or cancelled.

D: Duplicated means repeated, not awaiting completion.

Model response: Those times refer to different events. The requisition records collection at nine twenty, while the courier sheet records arrival at ten oh five. Please keep the two labels clear rather than replace one time with the other. If either source is questioned, we should ask the recording team to clarify that specific entry.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

05 | Referrals and appointment dependencies

Editing example: The referral is under review. Friday is Lia's next status check; no visit is booked.

A follow-up date should not be mistaken for an appointment.

Check 1: B - check

A: Attend applies to a visit, not checking status.

B: Correct. Check describes Lia's information task.

C: Complete promises a finished referral process.

D: Cancel would withdraw the request.

Check 2: B - scheduled

A: Submitted means sent for consideration.

B: Correct. Scheduled means a visit has been arranged.

C: Requested describes initiating the referral.

D: Dispatched describes the earlier sending action.

Model response: Referral F8 was sent Monday and received Tuesday. The specialist office is reviewing it, but no appointment has been booked. Friday is when Lia will check the status, not a visit date. We need a separate appointment confirmation before we can give you a visit time or tell you which site to attend.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

06 | Supporting questions during a clinical visit

Editing example: That phrase is unclear to you. I'll ask Dr Chen to explain it here before you leave.

The question gets a specific recipient and location.

Check 1: D - clarify

A: Abbreviating makes it shorter, not necessarily clearer.

B: Filing stores the document rather than explaining its wording.

C: Withdrawing removes something rather than explaining it.

D: Correct. Clarify means make the intended meaning clear.

Check 2: D - teach-back

A: A referral requests another service.

B: Discharge approval is a separate decision.

C: A diagnosis identifies a condition.

D: Correct. Teach-back checks the explanation through the patient's understanding.

Model response: Dr Chen, Jules has a question about a phrase in the after-visit summary and would like it explained before leaving. I have not reinterpreted the instruction. Jules prefers to discuss it here rather than at reception. Could you return to the room and check the explanation with Jules before we finish the visit?

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

07 | Telephone concerns and clinical escalation

Editing example: The booking doesn't assess this new concern. I'll connect you with Dana for clinical review.

A calendar entry cannot establish clinical urgency.

Check 1: D - triage

A: Billing concerns payment rather than assessing clinical urgency.

B: Attendance confirmation checks whether someone will come.

C: Registration identifies the patient and visit.

D: Correct. Triage assesses urgency and response; the assistant is not performing it here.

Check 2: D - warm transfer

A: A reminder notice does not hand over a live call.

B: A missed call is an unanswered contact.

C: A booking amendment changes a visit.

D: Correct. A warm transfer includes an introduction to the receiver.

Model response: I can't determine from an appointment booking whether this new concern can wait until next week. Dana is available through our clinical-call route now, and I will connect you so she can assess it. I will pass on your question about timing without describing the concern as routine or offering reassurance about its importance.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

08 | Records, corrections, and follow-up ownership

Editing example: I gave the wrong day. Your confirmed discussion is Thursday at two with Dr Chen.

A specific correction is more useful than a vague apology.

Check 1: D - confirmed

A: Cancelled contradicts the existing booking.

B: Provisional would describe a tentative booking.

C: Requested would mean asked for, not booked.

D: Correct. Confirmed matches the verified schedule.

Check 2: A - outstanding

A: Correct. Outstanding means the contact remains to be done.

B: Completed falsely says contact occurred.

C: Duplicated means repeated, which is not stated.

D: Withdrawn would mean the task was cancelled.

Model response: I'm sorry my message gave Tuesday for your results discussion. The confirmed appointment is Thursday at two with Dr Chen. I understand you changed your work plans because of my message. I'll correct that message in the record and check that you have the Thursday details; I cannot interpret the results during this scheduling call.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

Give feedback that helps

Score each criterion 0, 1, or 2. This is a practice rubric, not a professional qualification or CEFR test. Do not award or remove points for a particular accent.

Meaning and accuracy

Keeps the case facts accurate; clearly separates confirmed and unknown information.

Clarity and language

Uses understandable sentences and explains specialist terms when the listener needs it.

Interaction and tone

Responds to the other person's question, checks understanding, and uses an appropriate tone.

Task completion

Makes the requested action or unresolved question clear without inventing authority or facts.

Use the same scale for each criterion

0 - Not yet: The reader or listener cannot yet recover the intended meaning or action.

1 - With support: The message works after a prompt, clarification, or revision.

2 - Independently: The message is clear and accurate without a prompt.

Feedback sequence

First, identify a successful phrase. Next, identify one point where meaning or accuracy needs work. Offer one useful language correction, allow a repeat, and compare the two attempts. A total out of eight describes this performance only; do not translate it into a proficiency level.

Final challenge

Choose a case not rehearsed today. The learner gives a one-minute response, answers two follow-up questions, and switches roles and repeats the exchange. Add the lesson's second-round challenge. Compare the result with an earlier attempt using the four criteria.

Use the language responsibly

Every patient, symptom report, date, measurement context, and clinic arrangement is fictional language practice. This is not clinical, diagnostic, triage, medication, specimen-collection, privacy-law, or treatment training. Duties depend on local rules, training, delegation, and supervision. Qualified clinicians assess urgency and make care decisions; use the workplace's actual emergency and escalation procedures, not these scripts, for real concerns.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

U.S. Bureau of Labor Statistics: Medical Assistants

<https://www.bls.gov/ooh/healthcare/medical-assistants.htm>

American Association of Medical Assistants: Medical Assisting

<https://www.aama-ntl.org/medical-assisting>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.

Extend your practice with AI

Use the following prompts after your own first attempt. Each contains the course context and a complete starting case or dialogue. Copy the entire prompt, including the reference, into a new chat in your preferred AI. You can also use the links to copy it from the website.

Choose the right kind of help

A role-play prompt makes the AI wait for your replies. A new-dialogue prompt asks for a complete professional script. Vocabulary and grammar prompts move from explanation to your own attempts.

Choose any lesson or dialogue online

The course prompt workshop has four practice goals and B1 support, B2 independent and C1 stretch settings. These are practice choices, not assessed proficiency levels. Select the same lesson number or dialogue title you used in this guide.

[Open this course's AI prompt workshop](#)

Keep practice useful

Use fictional or anonymized details. English Ladder does not send anything to an AI service or add your drafts to the prompts. Your chosen service's terms and any usage charges apply. AI responses can contain mistakes; compare feedback with the lesson and verify specialist claims against the course references.

Each starter prompt uses B2 practice. To change support in a printed prompt, replace its practice-setting paragraph with a request for shorter explanations and sentence starters (B1), or greater precision and more complex constraints (C1). Keep the professional facts unchanged.

Changing the example

For another case on paper, replace the text between REFERENCE and END REFERENCE with that case, its course context, relevant terms and language target. Include the full exchange if practicing a dialogue. Do not assume your AI can access this PDF or a link. The website makes this substitution for you.

Prompt edition: 2026-09-30. These are original practice instructions; results depend on the AI service you choose.

Practice grammar in context

START PROMPT / COPY THROUGH END PROMPT | ONLINE VERSION

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Medical Assisting English

Professional audience: Medical assistants and outpatient clinic support staff

Scope: Every patient, symptom report, date, measurement context, and clinic arrangement is fictional language practice. This is not clinical, diagnostic, triage, medication, specimen-collection, privacy-law, or treatment training. Duties depend on local rules, training, delegation, and supervision. Qualified clinicians assess urgency and make care decisions; use the workplace's actual emergency and escalation procedures, not these scripts, for real concerns.

Lesson 01: Clarifying the visit at check-in

Original fictional case: The schedule says annual review. Patient Alex expected a visit about a recent concern. The assistant cannot decide whether the appointment type is suitable. Nurse Dana can clarify the visit plan. Alex prefers Alex, while the verified record says Alexandra.

Language workshop: Acknowledge the expectation before checking the booking

Communication goal: Describe a visit mismatch without dismissing the patient's reason for attending.

Language guidance: A booking label and the patient's chief concern serve different purposes. Ask what the patient expected, preserve their wording, and leave decisions about the visit's clinical suitability to the team.

Useful frames (complete the gaps with case facts): The schedule says ____; you expected _____. / How would you describe your main concern today? / I can address you as _____ while verifying the record under _____.

Vocabulary: chief concern: The main reason a patient gives for seeking care at the current visit. / intake

form: A clinic document used to gather patient information before or during a visit. / patient identifier: A

detail used with the clinic's verification process to distinguish one patient from another. / preferred name:

The name a patient asks staff to use when addressing them.

Speaking task: Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Alex: explain the different expectation and ask staff to use your preferred name.

Guided response task: Complete a workplace dialogue by selecting one of four supplied options. Compare the feedback and say the completed exchange aloud. Do not write an open-ended message.

Optional second-round challenge: Ask whether changing the display name also changes the verified identity record.

Model for comparison AFTER my attempt, not a response to give me first: Alex, the schedule describes an annual review, but you expected to discuss a recent concern. I will tell Dana about that difference so she can clarify the visit plan with you. I can use Alex when addressing you while keeping the verified record linked to Alexandra. I haven't confirmed a change to the appointment type.

END REFERENCE

TASK: Choose grammar that preserves the meaning

1. Use the lesson's language workshop, or one actual sentence from the dialogue, to explain a single grammar distinction in no more than 90 words. Show how it affects timing, certainty, responsibility or politeness.
2. Give one brief occupational exchange containing a gap and four complete options A-D. Supply enough context for exactly one answer. Keep distractors plausible; distinguish grammar errors from wording that changes the facts. Stop and wait.
3. Explain why each choice fits or fails, then ask me to read the completed exchange aloud. Do not request a draft, original written sentence or essay.
4. Repeat with three new exchanges, one at a time, moving from recognition to a more subtle meaning contrast. Do not copy the published checks. Label invented variations and do not turn uncertainty into a promise.
5. End with a short contrast table and two supported spoken turns using supplied frames. Accept accurate alternatives in oral practice; acknowledge and replace ambiguous multiple-choice items.

END PROMPT